

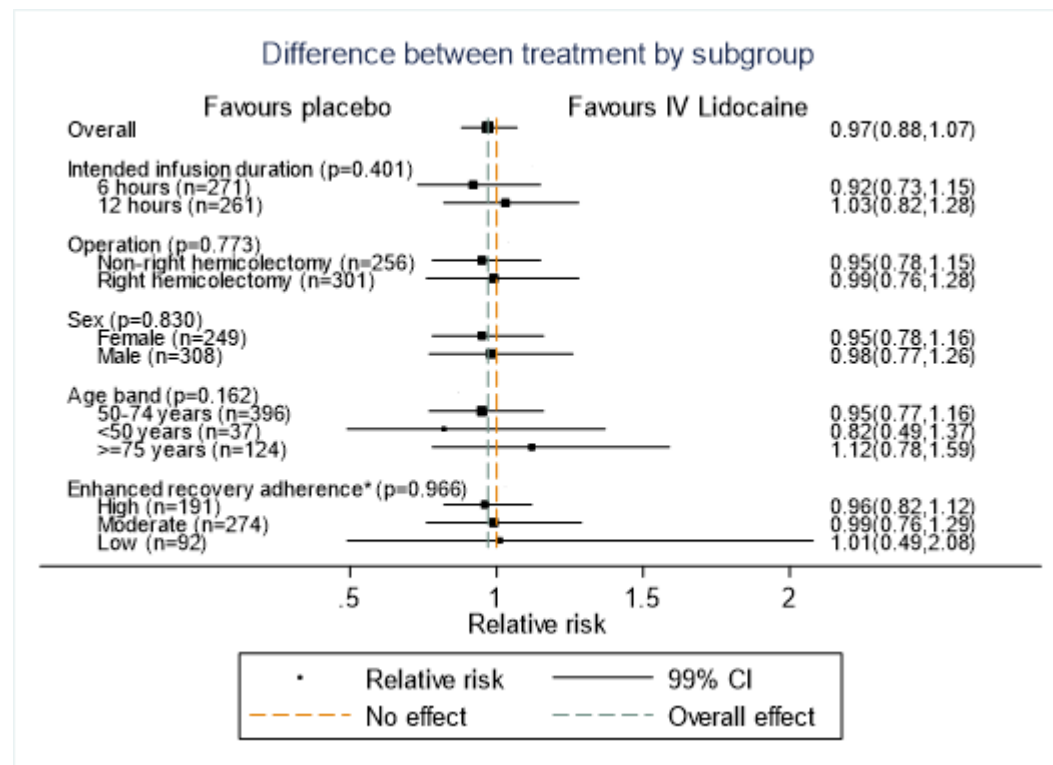
Supplemental Online Content

Paterson H, Vadiveloo T, Innes K, et al. Intravenous lidocaine for gut function recovery in colonic surgery: a randomized clinical trial. *JAMA*. doi:10.1001/jama.2024.23898

eFigure. Prespecified Subgroup Analysis of Primary Outcome

This supplemental material has been provided by the authors to give readers additional information about their work.

eFigure. Prespecified Subgroup Analysis of Primary Outcome (return of gut function by GI-3 definition at 72 hours after surgery)



The numbers on the right are the within-subgroup relative risks and 95% confidence interval. The p-values on the left of the graph are the p-values for interaction.

*Enhanced Recovery adherence subgroup analysis was pre-planned, however the definition of high, moderate and low were not pre-defined. Adherence was defined from the following criteria: Postoperative nausea prophylaxis prescribed regularly; No nasogastric tube placed at operation; Total IV fluids in first 24 hrs from start of anaesthesia <4litres (any fluid type); Preoperative carbohydrate loading drink given; Day 1 mobilisation target achieved; Food offered day 1; Supplement drinks given on day of surgery; IV fluids discontinued within 48 hours of start of operation; Urinary catheter removed within 48 hours of start of operation; Mobilisation target achieved (out of bed for at least 4 hours day 2). "High" adherence was defined if a patient met 7 or more criteria and "low" adherence defined if a patient met less than 5 criteria.